

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

July 14, 2025

MEMORANDUM FOR: (b)(7)(F),(b)(6) (b)(7)(C) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(7)(F),(b)(6) (b)(7)(C)

THROUGH: CAPT (b)(7)(F),(b)(6) (b)(7)(C), MD
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FROM: CAPT (b)(7)(F),(b)(6) (b)(7)(C), RN
Investigator
ICE Health Service Corps

(b)(7)(F),(b)(6) (b)(7)(C)

SUBJECT: Mortality Review Report of Findings
Nhon NGUYEN, (b)(6),(b)(7)(C)

Executive Summary:

On April 16, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Nhon NGUYEN, (b)(6),(b)(7)(C). ICE detained Mr. NGUYEN, a 55-year-old Vietnamese male, at El Paso Service Processing Center (EPSPC), in El Paso, Texas (TX), from February 25 to April 16, 2025, the date of his death. Mr. NGUYEN's autopsy results are pending.

Prior to his detention, on July 14, 1983, Mr. NGUYEN entered the United States as a Legal Permanent Resident. On February 23, 2025, ICE Enforcement and Removal Operations (ERO) El Paso generated a Field Operations Worksheet for the apprehension of Mr. NGUYEN. On

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February 24, 2025, Mr. NYGUYEN reported as scheduled to the ERO El Paso Albuquerque Sub-Office Non-Detained Unit. Upon consultation with ERO's Removal Management Division, ERO El Paso arrested Mr. NGUYEN and detained him at EPSPC, where he remained until April 16, 2025.

On February 25, 2025, a licensed vocational nurse (LVN) completed Mr. NGUYEN's intake assessment and documented his altered mental status (AMS), disorientation (oriented only to self), inability to answer any questions, including his medical history, abnormal gait, and weakness, and normal vital signs. The LVN contacted an advanced practice provider (APP) regarding her inability to assess Mr. NGUYEN's cognitive status (using an interpreter), and the APP referred Mr. NGUYEN to the University Medical Center (UMC), in El Paso, TX, for further evaluation. IHSC staff at EPSPC notified ERO El Paso they did not have the on-site resources and equipment necessary to provide for Mr. NGUYEN's medical needs and determined he would require a facility that could provide a higher level of care to assist with his activities of daily living (ADLs).

On February 26, 2025, UMC transferred Mr. NGUYEN to the Long-Term Acute Care (LTAC) Hospital, in El Paso, TX, for continued care related to his AMS, confusion, and need for assistance with ADLs.

Between February 27 and April 5, 2025, Mr. NGUYEN's status steadily declined, and EPSPC medical staff continued to transfer him, due to his continued need for a higher level of care.

On April 6, 2025, an APP completed Mr. NGUYEN's initial health assessment, reviewed his off-site disposition from LTAC Hospital showing a diagnosis of acute encephalopathy (a sudden disturbance in brain function), hyperactive delirium (a type of agitation) with possible vascular dementia (brain damage caused by multiple strokes). The APP documented an inability to complete a thorough assessment due to his AMS and placed him in the medical housing unit (MHU) for observation.

On April 7, 2025, a registered nurse (RN) evaluated Mr. NGUYEN during MHU rounds, documented his continued AMS and needed assistance with ADLs. The RN contacted an APP, and the APP referred Mr. NGUYEN to LTAC Hospital for a higher level of care.

Between April 8 and April 15, 2025, LTAC Hospital staff reported Mr. NGUYEN's continued declining status, placing him on a BiPAP (positive airway pressure) machine due to respiratory distress, and the hospital staff prepared to transfer him to hospice for end-of-life care.

On April 16, 2025, at 1:34 p.m., LTAC Hospital medical staff pronounced Mr. NGUYEN deceased.

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Mortality Finding:

As a result of Mr. NGUYEN's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined EPSPC medical staff provided Mr. NGUYEN medical care within the safe limits of practice and concurred there were no deviations beyond safe limits of practice that posed a future risk of adverse outcomes.

The MRC convened on June 20, 2025, and consisted of the following IHSC members: Doctor (b)(7)(F),(b)(6) (b)(7)(C), staff physician, physician reviewer; Lieutenant Commander (LCDR) (b)(7)(F),(b)(6) (b)(7)(C), nurse practitioner, supervisory APP, APP reviewer; Captain (CAPT) (b)(7)(F),(b)(6) (b)(7)(C), RN, western regional nurse manager, nurse reviewer; and LCDR (b)(7)(F),(b)(6) (b)(7)(C), RN, risk management program manager, risk management reviewer.

The following summary of events was based on 1) Mr. NGUYEN's EPSPC medical records 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. NGUYEN's EPSPC detention file; 5) staff interviews conducted by CAPT (b)(7)(F),(b)(6) (b)(7)(C) via Microsoft Teams; 6) applicable EPSPC policies and procedures; and 7) applicable accreditation standards.

Summary of Events

On February 25, 2025, an LVN completed Mr. NGUYEN's intake assessment and documented his AMS, disorientation (oriented only to self), inability to answer any questions including his medical history, abnormal gait and weakness, and normal vital signs. The LVN contacted an APP regarding her inability to assess Mr. NGUYEN's cognitive status, and the APP referred Mr. NGUYEN to UMC, in El Paso, TX, for further evaluation.

On February 26, 2025, UMC transferred Mr. NGUYEN to LTAC Hospital, in El Paso, TX, for continued care related to his AMS, confusion, and need for assistance with ADLs.

Between February 27 and March 12, 2025, LTAC Hospital staff reported Mr. NGUYEN's intermittent confusion and continued AMS of an unknown origin, for which they referred him to psychiatry, and noted his physical deconditioning, for which they referred him to occupational and physical therapy. LTAC Hospital nursing staff noted Mr. NGUYEN's inadequate oral intake related to his AMS but documented no significant changes to his health status.

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On March 13, 2025, an RN completed Mr. NGUYEN's off-site return from LTAC Hospital, reviewed his discharge disposition showing a computed tomography (CT) scan impression of brain atrophy (condition that involves the loss of brain cells) and small-vessel ischemic changes of the brain (damage to the blood vessels in the brain), documented his AMS and stable vital signs, contacted an APP, and received orders to admit him to the MHU for observation.

- On the same day, an RN completed Mr. NGUYEN's MHU admission, documented his inability to answer questions, continued AMS, his inability to feed himself or use the bathroom independently, and his need for full assistance with ADLs. The RN contacted an APP regarding his high-level care needs, and the APP referred him to UMC for further evaluation.

On March 16, 2025, UMC transferred Mr. NGUYEN to LTAC Hospital, for continued care related to his AMS, confusion, and need for assistance with ADLs.

Between March 16 and April 5, 2025, LTAC Hospital staff provided Mr. NGUYEN maximum ADL assistance, referred him for appropriate in-house specialty care, ordered diagnostic imaging, assessments, and lab work, as needed, and noted his continued altered mental status and physical deconditioning.

On March 19, 2025, a supervisory detention and deportation officer emailed a docket officer to conduct a case review due to IHSC's request to release Mr. NGUYEN from custody because of his need for a higher level of care than IHSC at EPSPC could provide.

On March 21, 2025, a deportation officer (DO) unsuccessfully attempted to obtain a family member or friend point-of-contact from Mr. NGUYEN and documented Mr. NGUYEN's inability to communicate or hold a conversation.

On March 31, 2025, an attorney successfully filed a Notice of Entry of Appearance as Attorney or Accredited Representative, Form G-28, on behalf of Mr. NGUYEN, and a DO contacted Mr. NGUYEN's attorney to obtain next-of-kin (NOK) information.

On April 2, 2025, Mr. NGUYEN's attorney notified ERO El Paso of Mr. NGUYEN's family members' inability to take custody of or care for him.

On April 5, 2025, an RN completed Mr. NGUYEN's off-site return from LTAC Hospital, documented his current AMS and stable vital signs, and placed him in the MHU for observation.

On April 6, 2025, an APP completed Mr. NGUYEN's initial health assessment and reviewed his off-site disposition from LTAC Hospital, showing a diagnosis of acute encephalopathy and hyperactive delirium with possible vascular dementia. The APP documented an inability to complete a thorough assessment due to his AMS and placed him in the MHU for observation.

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On April 7, 2025, an RN evaluated Mr. NGUYEN during MHU rounds, and documented his continued AMS and needed assistance with ADLs. The RN contacted an APP, and the APP referred Mr. NGUYEN to LTAC Hospital for a higher level of care.

Between April 8 and 11, 2025, LTAC Hospital staff continued to report Mr. NGUYEN's diagnosis of AMS, dementia, agitation, buttock wound (stage II pressure ulcer), and maximum assistance required for all ADLs, ordered lab work, administered IV fluids and medications, as needed, monitored his vital signs, and evaluated Mr. NGUYEN for hospice care (end of life care) for declining dementia.

On April 12, 2025, LTAC Hospital staff evaluated Mr. NGUYEN for agonal breathing (shallow breathing when the brain is not getting enough oxygen) and decreased activity level, placed him on supplement oxygen, reviewed lab work, administered IV fluids and medications, as needed, monitored his vital signs, started the process of obtaining a Do-Not-Resuscitate (DNR) order due to a poor prognosis, and continued to provide 1:1 palliative care with maximum ADL assistance.

- On the same day, an LTAC Hospital physician signed a DNR order for Mr. NGUYEN due to his terminal condition.

Between April 13 and 15, 2025, Mr. NGUYEN became non-responsive. LTAC Hospital staff started him on a BiPAP machine (non-invasive breathing device) for continued difficulty breathing, continued to provide complete 1:1 palliative care, and recommended hospice placement, pending approval.

On April 15, 2025, ERO El Paso attempted to contact Mr. NGUYEN's attorney and requested assistance obtaining NOK information to discuss Mr. NGUYEN's health status; however, the attorney did not immediately respond.

On April 16, 2025, at approximately 6:00 a.m., LTAC Hospital medical staff noted Mr. NGUYEN remained non-responsive to any stimuli, had fixed pupils and diminished lung sounds, and his body was cold to touch.

- On the same day, at 1:34 p.m., LTAC Hospital medical staff announced that Mr. NGUYEN deceased.
- On the same day, at 4:28 p.m., ERO Miami obtained NOK information from Mr. NGUYEN's attorney, and ERO Miami notified Mr. NGUYEN's NOK of his passing.

Mr. NGUYEN's final autopsy report is pending.

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The MRC used the following detention standards for this review: Performance-Based National Detention Standards, 2011 with 2016 Revisions. The MRC identified the following program strengths and recommendations during this review.

Strengths

The MRC identified the following strengths: 1) EPSPC medical staff appropriately evaluated and monitored Mr. NGUYEN's medical conditions and made the appropriate referrals to a higher level of care.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Systems Support Division (HSSD), DAD of Clinical Services Division (CSD), and DAD of Health Care Compliance Division (HCCD).
- The IHSC DAD of HSSD will share these findings with EPSPC's health services administrator (HSA) for review, dissemination to ICE and EPSPC's facility administrator, and any action(s) deemed appropriate.
- The IHSC DADs of CSD and HCCD will share these findings with the respective IHSC CSD and HCCD units for review and any action(s) deemed appropriate.
- EPSPC's HSA and the respective IHSC CSD and HCCD units will ensure a corrective action plan, if applicable, implemented and sustained.

End of report.